

PRINTABLE HOSPITAL-FAMILY REFERENCE

The Hospital Discharge & Medicare Quick Guide

A plain-English printable reference for families before discharge, rehab, home support, long-term care, or a confusing bill. Written from a healthcare-worker perspective by Andrew Ciccarelli, RN, BSN. Educational only. Verify with official sources, plan documents, agency notices, SHIP, or a qualified professional.

Status	Payer	Notice	Deadline	Next call
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Step 1: Status What is the official status?	Step 2: Payer Which coverage path applies?	Step 3: Notice What written document explains it?	Step 4: Deadline What changes today or this week?	Step 5: Next call Who can verify in writing?
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DIRECT ANSWER

Use this guide to slow the decision down, identify the payer, ask for the written rule, and avoid signing, leaving, or paying before the discharge story is clear.

CLEAR CARE PATH

Work the question in this order.

1 Situation What happened?	2 Payer Who might pay?	3 Document What explains it?	4 Next step Who verifies it?
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Who this is for

- Family at discharge:** You are being told home, rehab, SNF, home health, equipment, or private pay may be next.
 - Patient with a bill:** You have a hospital, facility, doctor, agency, ambulance, or supplier bill that does not match what you expected.
- Caregiver on the phone:** You need short questions, not a Medicare textbook.
 - Healthcare worker helping family:** You know the care plan, but the payment and paperwork path still feels unclear.

First four checks

- Status:** Is the patient inpatient, observation, outpatient, discharged, or already transferred?
 - Document:** What notice, order, authorization, MSN, EOB, bill, or denial explains the answer?
- Payer:** Is the issue Original Medicare, Medicare Advantage, Medicaid, employer insurance, supplemental coverage, or private pay?
 - Deadline:** Is there an appeal, discharge, authorization, or payment deadline that needs action today?

What this means in real life

- Do not rely on hallway answers:** Ask who can verify the answer in writing.
 - Do not ignore notices:** Discharge, denial, and coverage-ending notices can start short response windows.
- Do not treat “medically needed” as the same as payer approval:** A care need and a payment rule are separate questions.
 - Do not pay a confusing bill first:** Match it to the processed claim paperwork before paying.

Source note: Built from Medicare.gov, Medicaid.gov, CMS discharge/appeal resources, SHIP resources, and the official source map at the end of this guide.

AVOID COSTLY ASSUMPTIONS

The 5 Misunderstandings

Use this page to ask better questions before signing, paying, enrolling, or appealing.

DIRECT ANSWER

Most discharge confusion comes from mixing up care need, hospital status, authorization, program rules, and the final bill. Treat each one as a separate checkpoint.



Five things to separate

- Need vs payment:** A patient may need help, but the payer still applies rules, documents, networks, medical-necessity standards, and timing.
- Recommendation vs authorization:** A doctor, therapist, or case manager recommendation is not the same as a plan authorization.
- Processed claim vs bill:** An MSN or EOB tells the claim story; a bill asks for payment. The numbers should be compared before you pay.
- Hospital bed vs hospital status:** Sleeping in the hospital does not by itself prove inpatient status.
- Medicare vs Medicaid:** Medicare is usually the health-insurance question; Medicaid may become the long-term services/supports or low-income assistance question.

Ask today

- Rule:** “What rule, notice, or plan document are you relying on?”
- Timing:** “What deadline applies if we disagree or need review?”

- Proof:** “Can you send or print the document that shows this?”
- Backup:** “What happens if authorization is delayed, denied, or ends before the patient is safe?”

What this means in real life

- Before transfer:** Ask whether the receiving facility, agency, or supplier is approved, participating, or in-network for the payer.
- Before discharge home:** Ask what skilled services, equipment, medications, and follow-up appointments are actually ordered.
- Before agreeing to private pay:** Ask what changed, what was denied or not authorized, and what written appeal/review path exists.

Source note: Built from Medicare.gov coverage/cost materials, CMS observation-status materials, Medicaid.gov eligibility/LTSS materials, SHIP resources, and Medicare appeal resources.

STATUS CHECK

Inpatient vs Observation

Use this page to ask better questions before signing, paying, enrolling, or appealing.

DIRECT ANSWER

Hospital status can affect billing and some downstream care rules. Ask for the official status and the date/time it started instead of relying on the room, length of stay, or whether the patient slept overnight.

CLEAR CARE PATH

Work the question in this order.

1
Status
Inpatient or observation?

2
Timing
When did it start?

3
Notice
Was one given?

4
Impact
Could it affect rehab?

Know the difference

- Inpatient:** A hospital admission under an inpatient order; ask when the inpatient order began.
- Outpatient:** Services may happen inside the hospital without an inpatient admission.
- SNF question:** Under Original Medicare, skilled nursing facility rules can depend on a qualifying inpatient hospital stay, with details and exceptions to verify.
- Observation:** An outpatient hospital service, even when the patient stays overnight in a hospital bed.
- Notice:** Medicare rules include observation-status notice requirements for certain patients.

Ask before discharge

- Current status:** “Is the patient inpatient, outpatient, or observation right now?”
- Notice:** “Was a MOON or other status notice issued, and can we have a copy?”
- Who can verify:** “Can utilization review, case management, or billing confirm this in writing?”

- Timing:** “What date and time did inpatient admission begin, if it did?”
- Rehab impact:** “Could this status affect SNF, rehab, or post-acute coverage rules?”

What this means in real life

- Example:** A patient can spend multiple nights in the hospital and still have outpatient/observation status for Medicare billing purposes.
- If confused:** Ask the hospital to explain the status in plain English and identify the appeal or review path if one exists.
- Practical step:** Save the status notice, admission order timing, discharge paperwork, and any plan authorization messages together.

Source note: Built from CMS and Medicare.gov materials on inpatient vs outpatient/observation status, MOON notice materials, and Medicare.gov skilled nursing facility materials.

REHAB REVIEW

Rehab and Skilled Nursing Facility Care

Use this page to ask better questions before signing, paying, enrolling, or appealing.

DIRECT ANSWER

Rehab may be appropriate medically, but payment can depend on the setting, documented skilled need, hospital status, facility participation/network, timing, authorization, and written notices.

CLEAR CARE PATH

Work the question in this order.

1
Skilled need
What is documented?

2
Facility
Is it approved?

3
Authorization
Approved or pending?

4
Backup
What if it ends?

Sort the setting

- **SNF:** Skilled nursing facility care is different from long-term custodial nursing-home care.
 - **Home health:** Skilled services at home under specific rules; not the same as round-the-clock home help.
 - **Custodial care:** Ongoing help with bathing, dressing, meals, toileting, supervision, or transportation may follow different rules.
- **Inpatient rehab:** A more intensive rehab setting with different criteria and documentation.
 - **Outpatient therapy:** Therapy after discharge without facility admission.

Ask before transfer

- **Qualifying stay:** “Does the patient have the hospital status/timing needed for this pathway, if applicable?”
- **Facility fit:** “Is this facility participating, approved, or in-network for this payer?”
- **Discharge plan:** “What is the expected goal, reassessment date, and backup plan if coverage ends?”

- **Skilled need:** “What skilled nursing or therapy need is documented today?”
- **Authorization:** “Is authorization approved, pending, denied, or ending?”

Call script

- **Start:** “I am trying to verify the rehab payment path before transfer.”
- **Record:** Write down the name, department, date, reference number, and next deadline.

- **Ask:** “What rule applies, what document proves it, and what can we do if the answer changes?”

Source note: Built from Medicare.gov skilled nursing facility materials, Medicare.gov care comparison materials, Medicare Advantage coverage/appeal materials, CMS appeal resources, and the official source map.

HOME SUPPORT

Home Health vs Daily Home Help

Use this page to ask better questions before signing, paying, enrolling, or appealing.

DIRECT ANSWER

Home health, private-duty help, family caregiving, and custodial support are not the same. Ask what service is ordered, who provides it, how often it comes, and who is expected to pay.

CLEAR CARE PATH

Work the question in this order.



Know the difference

- **Home health:** Usually part-time or intermittent skilled care under specific rules, such as nursing or therapy ordered after discharge.
- **Custodial help:** Bathing, dressing, meals, toileting, supervision, and transportation are daily-living needs, not automatically skilled care.
- **Medicaid/aging resources:** Long-term home support may involve state Medicaid or local aging resources for eligible people.
- **Private-duty aide:** Help arranged privately or through an agency may be outside Medicare home health rules.
- **Family support:** A discharge plan may assume family availability; ask what happens if family cannot safely provide that level of help.

Ask before discharge home

- **Order:** “What home health, therapy, nursing, equipment, or medication support is actually ordered?”
- **Agency:** “Which agency accepted the referral, and when is the first visit?”
- **Frequency:** “How often will visits occur, and what happens between visits?”
- **Payment:** “Who is expected to pay for skilled visits, aides, equipment, supplies, and transportation?”
- **Backup:** “Who do we call if the patient is not safe at home after discharge?”

What this means in real life

- **A nurse visit is not 24-hour care:** Home health may be intermittent.
- **An aide is not automatically included:** Ask what aide services are approved, ordered, or private pay.
- **Equipment timing matters:** A walker, oxygen, bed, commode, or supplies may require documentation, supplier rules, delivery timing, or plan authorization.

Source note: Built from Medicare.gov home health materials, Medicare.gov durable medical equipment materials, Medicare.gov long-term care materials, Medicaid.gov LTSS materials, and SHIP resources.

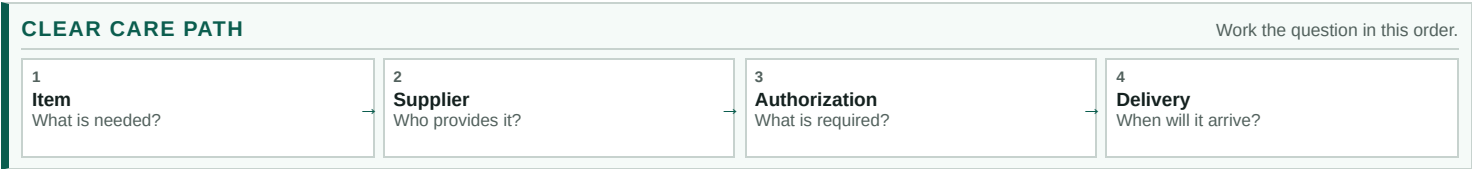
EQUIPMENT AND AUTHORIZATION

Equipment, Medications, and Plan Authorization

Use this page to ask better questions before signing, paying, enrolling, or appealing.

DIRECT ANSWER

Equipment, medications, suppliers, and plan authorization can delay a safe discharge. Verify the exact item, supplier, payer rule, authorization status, delivery timing, and backup plan.



Check the item

- DME:** Durable medical equipment can include items such as walkers, wheelchairs, oxygen-related equipment, hospital beds, commodes, or other ordered equipment depending on rules and documentation.
- Order:** Ask whether the prescription/order, diagnosis, face-to-face note, therapy note, or medical-necessity documentation is complete.
- Substitution:** Ask what safe alternative exists if the exact item is delayed or denied.
- Supplier:** The supplier may need to participate with Original Medicare or be in-network for a plan.
- Delivery:** Ask when and where the item will arrive, and whether discharge depends on it.

Ask the plan, supplier, or discharge team

- Authorization:** "Is authorization required, submitted, approved, denied, or pending?"
- Responsibility:** "What might the patient owe, and what document explains that?"
- Medication bridge:** "Will the patient have enough medication until follow-up?"
- Network:** "Is this supplier or pharmacy in-network or participating?"
- Appeal/review:** "If this is denied or delayed, who starts the appeal or reconsideration?"

What this means in real life

- Do not leave without timing:** If oxygen, mobility equipment, wound supplies, or key medications are needed, ask for the exact handoff plan.
- Call early:** Equipment and medication delays are easier to solve before discharge than after the patient is already home.
- Save proof:** Keep the order, delivery confirmation, authorization notice, denial notice, and supplier contact information.

Source note: Built from Medicare.gov durable medical equipment materials, Medicare.gov home health materials, Medicare.gov prescription/plan materials, CMS appeal resources, and Medicare Advantage plan decision materials.

MEDICARE VS MEDICAID

Long-Term Care, Custodial Care, and Medicaid

Use this page to ask better questions before signing, paying, enrolling, or appealing.

DIRECT ANSWER

Do not assume Medicare pays for ongoing custodial long-term care. Medicare and Medicaid answer different questions, and Medicaid long-term services/supports depend on state rules, eligibility, service type, and documentation.

CLEAR CARE PATH

Work the question in this order.

1

Care type

Skilled or custodial?

→

2

Program

Medicare or Medicaid?

→

3

State rules

What varies locally?

→

4

Before paying

Who should advise?

Sort the need

- **Short-term skilled care:** Rehab, therapy, skilled nursing, or post-acute care may be reviewed under Medicare or plan rules.
- **Medicaid LTSS:** Medicaid may be the relevant program for eligible people who need long-term services and supports.
- **Dual eligible:** A person can have both Medicare and Medicaid, but the exact help depends on program details.
- **Long-term custodial care:** Ongoing help with daily living may not fit Medicare's medical-care pathway.
- **State rules:** Income, assets, level-of-care rules, estate recovery, spousal protections, and covered settings can vary by state.

Ask before agreeing to private pay

- **Type of care:** "Is this skilled care, custodial care, assisted living, nursing home care, or home/community support?"
- **State help:** "Which state Medicaid agency, aging resource, or SHIP counselor should we contact?"
- **Planning:** "Should we speak with an elder-law attorney or qualified Medicaid planning professional before major financial decisions?"
- **Program:** "Is Medicare, Medicare Advantage, Medicaid, another payer, or private pay being discussed?"
- **Notice:** "Is there a denial, discharge notice, or coverage-ending notice we need to respond to?"

What this means in real life

- **Medicare vs Medicaid:** Medicare is usually the medical insurance starting point. Medicaid may become the long-term support question for eligible people.
- **Do not rely on one sentence:** Ask for the exact program name, agency, notice, and contact number.
- **Timing matters:** Medicaid applications and long-term care transitions can involve documentation requests and deadlines.

Source note: Built from Medicare.gov long-term care materials, Medicaid.gov eligibility materials, Medicaid.gov LTSS materials, Medicaid.gov estate recovery/spousal impoverishment materials, CMS Medicare-Medicaid coordination resources, and SHIP resources.

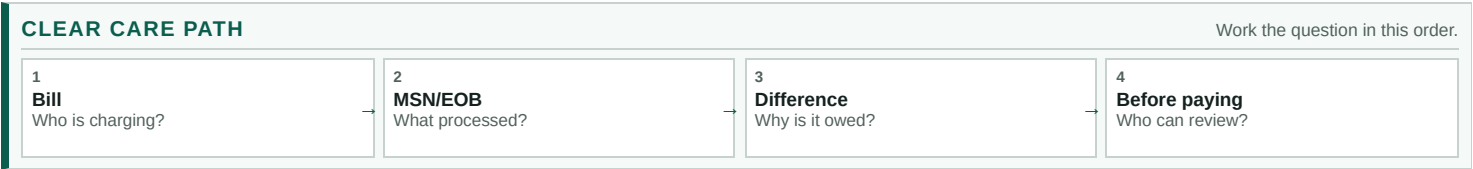
BILL CHECK

Bills, EOBs, and Medicare Summary Notices

Use this page to ask better questions before signing, paying, enrolling, or appealing.

DIRECT ANSWER

Do not pay a confusing bill until you match it against the processed claim story: provider bill, MSN, EOB, denial/appeal notice, adjustment, and any financial-assistance option.



Match the documents

- Provider bill:** The amount a hospital, doctor, facility, agency, ambulance company, or supplier says is owed.
 - EOB:** Explanation of Benefits from a private insurer or Medicare Advantage plan. It usually explains processing and is not itself a bill.
 - Financial assistance:** Hospitals may have financial-assistance or charity-care processes; ask before paying if the bill is difficult.
- MSN:** Medicare Summary Notice for Original Medicare claims. It explains processed claims and is not a bill.
 - Patient responsibility:** The amount left after deductible, copay, coinsurance, noncovered service, denial, adjustment, or plan processing.

Four-step bill check

- Match:** Date of service, provider, location, account number, and service description.
 - Question:** Ask why the patient owes the amount and whether coding, network, authorization, or coordination changed the result.
- Compare:** Bill amount vs MSN/EOB patient responsibility.
 - Escalate:** Ask about correction, appeal, rebilling, payment hold, itemized bill, financial assistance, or supervisor review.

Call script

- Billing office:** “I am not refusing to pay. I am trying to match this bill to the processed claim before payment.”
- Financial assistance:** “Can you screen this account for financial assistance or charity care before we set up payment?”
- Plan/Medicare question:** “Can you explain why this amount is patient responsibility and what document shows that?”

Source note: Built from Medicare.gov Medicare Summary Notice materials, Medicare.gov appeals materials, Medicare.gov cost materials, HealthCare.gov glossary definitions, and the official source map.

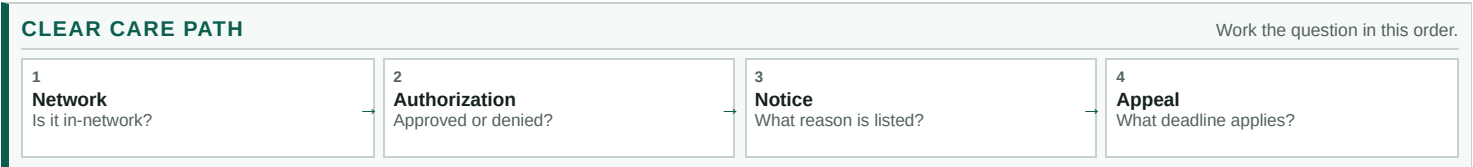
PLAN DECISIONS

Medicare Advantage, Prior Authorization, and Appeals

Use this page to ask better questions before signing, paying, enrolling, or appealing.

DIRECT ANSWER

Medicare Advantage is still Medicare, but plans can use networks, prior authorization, plan rules, notices, and appeal steps. Keep the tone neutral and ask for the written decision path.



Ask the plan or facility

- Network:** “Is this hospital, rehab facility, home health agency, supplier, or clinician in-network for this plan?”
- Notice:** “If the plan denied or ended something, what written notice explains the reason and deadline?”
- Appeal:** “Who files the appeal, what is the deadline, and can it be expedited if the patient may be unsafe?”

- Authorization:** “Was prior authorization required, submitted, approved, denied, or still pending?”
- Alternative:** “If this setting or supplier is not approved, what in-network or approved alternative is being offered?”

What this means in real life

- Not anti-plan:** The goal is not to assume the plan is wrong; the goal is to understand the plan rule, decision, deadline, and next step.
 - Write it down:** Record the representative name, call reference number, decision date, deadline, and fax/upload instructions.
- Get specifics:** “Pending authorization” is different from “denied,” “not medically necessary,” “out-of-network,” or “not a plan benefit.”

Quick appeal/review checklist

- Document:** denial, noncoverage, discharge, or coverage-ending notice.
 - Deadline:** exact date/time and whether expedited review is available.
- Clinical support:** therapy note, physician order, nursing note, wound/oxygen/mobility note, or unsafe-discharge concern.
 - Destination:** where to send the appeal and how to confirm receipt.

Source note: Built from Medicare.gov Medicare Advantage materials, Medicare.gov appeals materials, CMS managed-care appeal resources, plan decision resources, and SHIP resources.

CALL SCRIPTS

Scripts and Final Checklist

Use this page to ask better questions before signing, paying, enrolling, or appealing.

DIRECT ANSWER

When overwhelmed, use short questions, ask for the written source, and write down the name, department, date, reference number, document, and deadline.

CLEAR CARE PATH

Work the question in this order.

1

Call

Use one clear question.

→

2

Record

Write names and dates.

→

3

Escalate

Ask for the next level.

→

4

Save

Keep every notice.

Call scripts

- Hospital status:** “Can you tell me the patient’s official hospital status today and when that status began?”
- Rehab/SNF:** “What skilled need is documented, and what rule or authorization supports the next setting?”
- Equipment:** “Is the supplier participating or in-network, and when will the item be delivered?”
- Bill:** “Does this bill match the MSN, EOB, or processed claim record?”

- Discharge plan:** “What services, equipment, medications, and follow-up are ordered before discharge?”
- Home health:** “Which agency accepted the referral, when is the first visit, and what happens between visits?”
- Plan decision:** “Is this approved, denied, pending, or ending, and what written notice explains it?”
- Medicaid/LTSS:** “Which state agency or local aging resource should we contact for long-term support questions?”

Print-and-carry checklist

- Patient identifiers:** name, date of birth, Medicare/plan/member ID, Medicaid case number if applicable.
- Documents:** discharge paperwork, orders, medication list, authorization notice, denial notice, MSN, EOB, itemized bill.
- Open questions:** what is approved, what is pending, what might be owed, who verifies, and what happens if the plan fails.

- Contacts:** case manager, social worker, plan number, billing office, facility, agency, supplier, SHIP/state resource.
- Dates:** admission/status date, discharge date, service date, appeal deadline, authorization deadline, first home visit.

Last rule before signing or paying

- Pause:** “I need to verify the payer, document, and deadline before I agree.”
- Record:** Keep a simple call log with name, department, date, reference number, and exact next step.

- Escalate:** Ask for a supervisor, case manager, billing review, plan appeal team, SHIP counselor, state Medicaid agency, or qualified professional.
- Update:** If the answer changes, ask for the updated notice or document.

Source note: Built from the official source map below, Medicare.gov appeals materials, CMS plan decision resources, SHIP locator resources, Medicaid.gov resources, and guide-hub tool references.

OPTIONAL TRUST LAYER

Endnotes and Source Map

Official sources used to keep the short guide grounded without turning it into a textbook.

1. Medicare.gov — Skilled nursing facility care: <https://www.medicare.gov/coverage/skilled-nursing-facility-care>

3. CMS — Medicare Outpatient Observation Notice: <https://www.cms.gov/medicare/forms-notices/beneficiary-notices-initiative/moon>

5. Medicare.gov — Durable medical equipment: <https://www.medicare.gov/coverage/durable-medical-equipment-dme-coverage>

7. Medicare.gov — Medicare Advantage Plans: <https://www.medicare.gov/health-drug-plans/health-plans/your-health-plan-options>

9. Medicare.gov — Medigap: <https://www.medicare.gov/health-drug-plans/medigap/basics/how-medigap-works>

11. Medicare.gov — Medicare Summary Notices: <https://www.medicare.gov/basics/forms-publications-mailings/mailings/claims-and-appeals/medicare-summary-notice>

13. Medicaid.gov — Eligibility: <https://www.medicaid.gov/medicaid/eligibility/index.html>

15. Medicaid.gov — Estate recovery: <https://www.medicaid.gov/medicaid/eligibility/estate-recovery/index.html>

17. CMS — Coordination between Medicare and Medicaid: <https://www.cms.gov/medicare/coordination-benefits-recovery/medicare-medicaid-coordination>

19. Medicare.gov — SHIP help: <https://www.shiphelp.org/>

2. Medicare.gov — Inpatient or outpatient hospital status: <https://www.medicare.gov/basics/your-medicare-rights/your-protections/your-rights-in-the-hospital>

4. Medicare.gov — Home health services: <https://www.medicare.gov/coverage/home-health-services>

6. Medicare.gov — Long-term care: <https://www.medicare.gov/coverage/long-term-care>

8. Medicare.gov — Original Medicare: <https://www.medicare.gov/basics/get-started-with-medicare/get-more-coverage/your-coverage-options/compare-original-medicare-medicare-advantage>

10. Medicare.gov — Appeals: <https://www.medicare.gov/claims-appeals/how-do-i-file-an-appeal>

12. Medicare.gov — Medicare costs: <https://www.medicare.gov/basics/costs/medicare-costs>

14. Medicaid.gov — Long-term services and supports: <https://www.medicaid.gov/medicaid/long-term-services-supports/index.html>

16. Medicaid.gov — Spousal impoverishment: <https://www.medicaid.gov/medicaid/eligibility/spousal-impoverishment/index.html>

18. Medicare.gov — Medicare Savings Programs: <https://www.medicare.gov/basics/costs/help/medicare-savings-programs>

20. HealthCare.gov — Health insurance glossary: <https://www.healthcare.gov/glossary/>